

ArogyaRakshak — Presentation Script

Companion script for ArogyaRakshak_FYP_Presentation.pptx (10 slides)

How to use this document: Each section below matches one slide. It has (1) a one-line reminder of what's on screen, (2) a suggested speaker, (3) a word-for-word script written in plain, everyday language — no jargon, no reading out bullet points, just explaining it like you would to a family member. You don't have to say it exactly as written — read it a few times until it feels natural in your own words.

Suggested Speaker Rotation

Five slides each is too much for one person to sound natural — spread it across the team so everyone gets a turn.

Slide	Speaker
1 — Title	Viraj
2 — The Problem	Anurag
3 — Five Workflows	Sanjali
4 — How It Works (Upload → Act)	Raj
5 — Architecture	Arya
6 — App Design	Viraj
7 — ArogyaVault	Anurag
8 — Input → Process → Output	Sanjali
9 — Trust & Responsible AI	Raj
10 — Roadmap & Closing	Arya + Viraj (together)

Total suggested time: ~8–9 minutes (about 45–55 seconds per slide), leaving room for Q&A.

Adjust freely — swap people around based on who's more comfortable presenting which idea.

Slide 1 — Title: ArogyaRakshak

On screen: *Project name, tagline, team names, and a simple picture showing a patient with documents (bill, prescription, report, insurance) going into “ArogyaRakshak” and coming out the other side “understood → decided → acted on.”*

Speaker: *Viraj*

Script

Good morning/afternoon everyone. We are Team ArogyaRakshak — myself Viraj, along with Anurag, Sanjali, Raj, and Arya.

Let me start with a simple situation. Imagine you or someone in your family just got discharged from a hospital. What do you walk out with? A hospital bill you don't fully understand, a prescription, some medical reports, and if you have insurance, a pile of insurance paperwork you now have to deal with on your own.

Our project, ArogyaRakshak — which means “Health Protector” — is built to make that entire after-hospital journey simpler, smarter, and a lot more transparent. In short: you give us your documents, and we help you understand them, decide what to do, and actually take action — instead of you struggling through it alone.

Let us walk you through exactly how.

(~40 seconds)

Slide 2 — The Problem: The Healthcare Journey Doesn't End at Discharge

On screen: A chain — Discharge → Bill / Prescription / Reports → Insurance Claim → Possible Denial. Three problem words: Fragmented, Manual, Opaque.

Speaker: Anurag

Script

Most people think healthcare ends the moment you're discharged from the hospital. In reality, that's when the real headache begins.

You've got a bill sitting on one piece of paper, a prescription on another, medical reports somewhere else, and if you're insured, you now need to file a claim — and there's always a chance that claim gets rejected for a reason nobody explains to you clearly.

We noticed three things wrong with how this works today:

First, it's fragmented — all your documents and information are scattered across different papers, apps, and people. Nothing talks to each other.

Second, it's completely manual — you're the one typing out the same information again and again, checking bills by hand, comparing prices, filling the same forms for every single step.

And third, it's opaque — meaning it's hard to see through. Why was I charged this much? Am I eligible for a government scheme? Why did my insurance reject my claim? Most people simply don't know.

Our objective is simple: replace this entire fragmented, manual, and confusing process with one platform that does it all for you — you upload your documents once, we remember the details so you never repeat yourself, and everything becomes clear and actionable.

(~55 seconds)

Slide 3 — One Platform, Five Specialised Helpers

On screen: Five boxes — BillNyay, DawaCheck, SchemeSetu, DaaviSetu, BimaNyay — each with a plain question a patient might ask.

Speaker: Sanjali

Script

So how do we actually solve this? We didn't build one giant do-everything tool. Instead, we built five focused helpers inside one platform, each one answering a specific question a patient actually asks:

BillNyay answers: "Is my hospital bill correct?" — it checks your bill for overcharging.

DawaCheck answers: "Are my medicines fairly priced?" — it checks if you're paying more than you should for medicines.

SchemeSetu answers: "What government healthcare schemes might I qualify for?" — because a lot of people are eligible for free or subsidised treatment and simply don't know it.

DaaviSetu answers: "How do I prepare my insurance claim?" — it helps you put together everything an insurance company needs.

And BimaNyay answers: "My claim was denied — what can I do about it?" — it helps you understand why, and helps you file a proper complaint or appeal.

Now, a quick and honest note on boundaries, because this matters: this system is a decision-support tool — it helps you understand and act, it does not diagnose any illness, it does not automatically approve or reject anything on its own, and right now it works in English and depends on how clear your uploaded document is. We're not replacing doctors or hospitals or insurers — we're helping the patient navigate the paperwork around them.

(~55 seconds)

Slide 4 — How It Works: From Documents to Action in Five Steps

On screen: *A five-step flow — Upload → Understand → Store → Assist → Act.*

Speaker: *Raj*

Script

Let's make this concrete — here's exactly what happens, step by step, when you use ArogyaRakshak.

Step one — Upload. You simply take a photo with your phone camera, upload a PDF, or use the website. That's it — you do this once.

Step two — Understand. The system reads your document — think of it like the system "reading" the page the way you would, then picking out the important bits: the hospital name, the amount charged, the medicine name, whatever is relevant — and organises it neatly.

Step three — Store. That organised information gets saved safely into something we call ArogyaVault — basically your own personal, private health document locker.

Step four — Assist. This is where our shared "brain," which we call KADI, comes in. KADI remembers everything about your case and decides which of our five helpers — BillNyay, DawaCheck, and so on — should handle it. This is the part that saves you from repeating yourself. Upload once, and every relevant helper already knows your situation.

Step five — Act. Finally, you get a real, useful result — your bill gets audited, prices get compared, your eligibility gets checked, your claim gets prepared, or your denial gets challenged.

So the whole idea in one line: upload once, reuse that information everywhere, and actually get something done.

(~55 seconds)

Slide 5 — How the System Is Built (In Simple Terms)

On screen: Layered architecture diagram — user apps on top, ArogyaVault, then document/AI processing, then KADI in the middle connecting out to the five modules, with structured reports as the final output.

Speaker: Arya

Script

This slide looks a bit technical, so let's translate it into plain language — think of it like a small company with different departments, all reporting to one central manager.

At the top, you have what the patient actually sees and touches — the mobile app and the website. That's the reception desk.

Right behind that is ArogyaVault — your personal folder where all your uploaded documents are safely kept.

Then comes the reading and understanding layer — this is where the system looks at your document like a person would, picks out the text and numbers, and checks them against known rules — for example, government-approved hospital rate lists.

In the middle sits KADI — think of KADI as the office manager who never forgets a single detail about your case, and who knows exactly which department — sorry, which of our five helpers — should take over next. That's why we call it the “shared brain” of the system.

KADI then hands your case off to whichever specialist is relevant — BillNyay, DawaCheck, SchemeSetu, DaaviSetu, or BimaNyay. Importantly, these five helpers don't talk to each other directly — everything passes through KADI. This keeps each one simple, independent, and easy to test on its own, the same way different departments in a company each do their own job but report through one manager.

And finally, out comes a clean, structured result — an audit report, a price comparison, an eligibility answer, a ready claim package, or a grievance report — something you can actually read and use, not confusing hospital or legal language.

(~65 seconds)

Slide 6 — Designed Around the Patient (What the App Looks Like)

On screen: Conceptual mockups of a dashboard — document count, claims, recent activity feed showing things like “Hospital Bill — Audited”, “Insurance Claim — Processing”, “Claim Denial — Action Needed.”

Speaker: Viraj

Script

We didn't want to build something that only engineers would understand, so we designed the app around what a normal patient actually wants to see the moment they open it.

When you log in, you land on a simple dashboard — how many documents you've uploaded, how many claims are active, and a quick activity feed telling you exactly where things stand — for example, “your hospital bill has been audited,” “your insurance claim is being processed,” or “your claim denial needs your attention.”

There's one big, obvious button to scan a new document, and a workspace where you can browse everything — your bills, your claims, your reports, all in one place.

The important part: whether you're using it on your phone or on the website, it's the exact same experience — same backend, same KADI brain, same information — so you're never locked into one device.

Just to be transparent — what you're seeing here are early design mockups, meant to show our intended user experience, not final screenshots.

(~40 seconds)

Slide 7 — ArogyaVault: Your Personal Health Document Locker

On screen: ArogyaVault holding bills, prescriptions, reports, discharge summaries, insurance papers, claims, denials, scheme documents — with “Local” and “Cloud” storage, feeding into KADI.

Speaker: Anurag

Script

Let's talk specifically about ArogyaVault, because it's the foundation everything else is built on.

Think of it as your own personal digital locker for absolutely everything related to your health paperwork — bills, prescriptions, medical reports, discharge summaries, insurance policies, claims, denial letters, scheme documents — all of it, in one organised place, instead of scattered across WhatsApp chats, emails, and physical folders.

It's designed to be secure and available even without internet on your device, while also backing up safely to the cloud, so you never lose an important document.

One thing we want to be very clear about: ArogyaVault is not trying to replace your hospital's official medical records, or India's official health ID system, ABHA. It's simply your own personal copy, under your control, that we use to help you — nothing more.

And this is exactly what feeds into KADI, which then reuses this information across all five of our helpers, so you truly only upload something once.

(~45 seconds)

Slide 8 — From Documents to Decisions: What Each Helper Actually Does

On screen: A simple table — each of the five modules with its Input, its Process, and its Output.

Speaker: Sanjali

Script

Let's go quickly through what each of our five helpers actually takes in, what it does, and what you get back — think of it as five mini assembly lines.

BillNyay — you give it your hospital bill, it audits every line item against known fair rates, and you get back a clear bill audit report showing exactly where you may have been overcharged.

DawaCheck — you give it your prescription or medicine bill, it compares the price against benchmark rates, and you get a medicine price report.

SchemeSetu — you give it a bit of your background — your situation, income, condition — it checks that against government scheme eligibility rules, and you get an eligibility result telling you what you may qualify for.

DaaviSetu — you give it your policy and hospital documents, it prepares your claim step by step, and you walk away with a ready-to-submit claim package.

BimaNyay — you give it your policy, your claim, and the denial letter you received, it analyses exactly why the claim was denied, and gives you a grievance report you can use to appeal.

So the overall journey across all five is: verify your bill, compare your medicine prices, discover what schemes you qualify for, prepare your claim, and if needed, fight back against a denial.

(~50 seconds)

Slide 9 — Why You Can Trust It: Transparency & Responsible AI

On screen: *Four pillars — Transparency, Auditability, Privacy, Boundaries — plus a list of things the team actually measures/validates.*

Speaker: *Raj*

Script

Since this project deals with people's health and money, trust matters more than anything else — so let's talk about how we've tried to build that in.

Transparency — every result we give you is structured and clearly explained, not a vague “trust us” answer. And because we remember your context, you don't need to explain yourself again and again.

Auditability — our checks are based on clear, checkable rules, not a hidden black box, and everything can be traced back and reviewed by a real person if needed.

Privacy — your records stay under your control, stored securely, and only synced to the cloud in a controlled way.

And finally, our boundaries, which we take seriously: this system does not diagnose any medical condition, it does not approve anything automatically on its own, and it does not give you a legally binding recommendation. It supports your decision — it doesn't make the decision for you.

We're also actively thinking about how to measure and validate this properly — things like how accurately the system reads a document, whether its bill audits are correct, whether its eligibility

checks are correct, how complete a prepared claim is, and how good its denial analysis is — because a system like this is only as good as how well it's tested.

(~50 seconds)

Slide 10 — Where We Are, and Where We're Headed

On screen: *Three columns — Now / Next / Future — followed by a closing summary line.*

Speaker: *Arya + Viraj (together)*

Script

(Arya) Let's close with where we stand today, and where we're headed.

Right now, we have a working mobile and web app, ArogyaVault for document storage, KADI as our shared brain, and all five workflows — BillNyay, DawaCheck, SchemeSetu, DaaviSetu, and BimaNyay.

Next, our focus is on adding support for more Indian languages, making our document-reading even more accurate, covering more government schemes, and making the system smarter overall.

Looking further ahead, we want to connect with India's official digital health ecosystem — ABDM — and eventually work directly with hospitals and insurance companies, and expand what kinds of complaints and grievances we can help with.

(Viraj) So, to sum it all up in one sentence: ArogyaRakshak takes the messy, scattered pile of paperwork a patient is left with after a hospital visit, and turns it into one simple, AI-assisted journey — from documents, to understanding, to real action.

Thank you. We'd be happy to take any questions.

(~50 seconds)

Quick Delivery Tips

- **Don't read the slide out loud** — the audience can already see the words. Your job is to *explain* what it means, which is exactly what this script does.
- **Pause after each helper name** (BillNyay, DawaCheck, etc.) on Slides 3 and 8 — five names back-to-back is a lot to absorb; give the listener a beat.
- **Slow down on Slide 5** (architecture) — it's the most technical slide; the “office manager” analogy for KADI is there to keep it simple, lean on it.
- **Own your section** — read your part a few times in your own words rather than memorising it word-for-word; it'll sound more natural.
- **If asked “why do you need AI for this?”** — a simple answer: hospital bills, prescriptions, and insurance letters don't follow one fixed format, so a rigid program can't reliably read and understand all of them — that's exactly the kind of messy, real-world variation AI is good at handling.